



AI patronage nurse & digital twin for patients after hospital discharge

Solves problems #11 + #12

Polyclinics · private & dental clinics

Telegram + voice, in Uzbek



THE PROBLEM

Care stops at the hospital door

Deaths registered in Uzbekistan in 2025

177,100

104,500 (59%) from circulatory diseases — hypertension, heart failure, stroke: the chronic patients who go home after discharge with nobody updating their condition

No national statistic even tracks what happens after discharge.

Source: National Statistics Committee of Uzbekistan, 2025 data (reported by Kun.uz, Feb 2026)

01

No handoff after discharge

The family doctor is never notified. Follow-up breaks and the patient is left alone at home. (Problem #11)

02

Treatment without testing

Chronic patients get drugs with no way to see the effect on *their* body first. (Problem #12)

03

Private clinics lose patients after the visit

After surgery or an implant, nobody checks in. Complications are caught late, and patients go elsewhere.

OUR SOLUTION

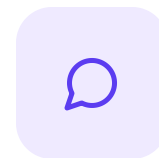
From discharge paper to continuous care at home



01

Discharge trigger

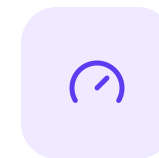
At discharge a **digital twin** is created and the family doctor gets an **"Active call"** task — confirm within 24 h.



02

AI nurse check-ins

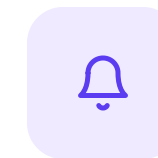
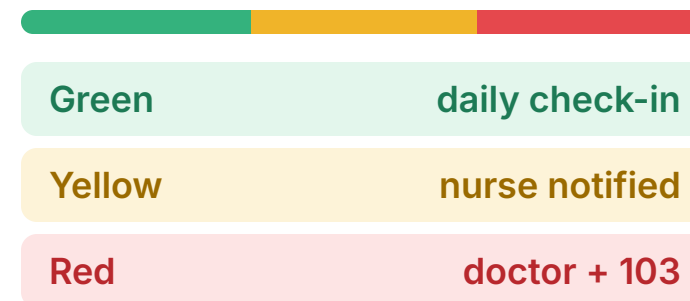
Daily on Telegram or by **voice call in Uzbek**: temperature, blood pressure, SpO₂, pulse, symptoms.



03

Risk zones

Fixed clinical rules (NEWS2-inspired) place the patient in a zone. **AI never decides alone.**



04

Action

No reply → AI calls → local nurse visits. Red zone → doctor alert, patient told to call 103.

🔄 Every step is logged — clinicians who follow protocol are protected.



AI follows world and local medical rules: WHO & international clinical guidelines (NEWS2, ESC, ADA) · Clinical protocols and standards of the Ministry of Health of Uzbekistan · ICD-10 diagnosis codes



PRODUCT DEMO

The doctor's view: a living digital twin

📁 Patient data comes from DMED and D-Clinics — we integrate with them, so doctors never re-enter data.

Shifokor paneli · Raqamli egizak

I10 Arterial gipertenziya

E11 · 2-tur qandli diabet · statsionardan chiqarilgan: 18-sentabr

EKG

Qon

Glyukoza

Xulosa

Barcha tahlillar (8)

AI maslahatlari

1/3

Bosim 3 kundan beri ko'tarilmoqda.
Amlodipin dozasini ko'rib chiqish tavsiya etiladi.

Egizakda sinab ko'rish →

Mutaxassislar xulosalari

DK Dr. Dilnoza Karimova

21-sen · Kardiolog

Ko'rik

MT Malika Tosheva

24-sen · Patronaj hamshira

Uyga tashrif

Butun tana

Yurak-qon tomir

Nafas

Organlar

1

2

Yurak · 2 ta signal

Tahlil tayinlash

Egizak simulyatsiyasi · 5 yil

Joriy davolash

Doza o'zgarsa

SCORE2 asosida · yakuniy qaror shifokorda

18%

11%

Oxirgi javob: bugun 08:41 · Telegram

CARE AI

AI nurse & digital twin after discharge

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TAM · \$31.8B

SAM · \$275M

SOM · \$25M

SIZE OF MARKET

From Uzbekistan to the world

TAM

\$31.8B · World Global remote patient monitoring market (2025)
→ \$66.2B by 2031, 12.8% CAGR

SAM

~\$275M · MENA + Central Asia Middle East & Africa RPM
\$225M (2025, 17% CAGR) + Central Asia ~\$50M

SOM

~\$25M / yr · Uzbekistan 7.8M hospital patients / yr × ~\$3.15
blended price × 1 month of monitoring

GOAL

\$1.8M ARR by 2029 ≈ 7% of the Uzbekistan market

Sources: MarketsandMarkets, Remote Patient Monitoring Market 2026–2031; Mordor Intelligence, MEA Remote Patient Monitoring Market 2025–2030; stat.uz (2024 hospital patients). Central Asia (Uzbekistan scaled by population) and Uzbekistan SOM are our bottom-up estimates.

WHY US? — AN HONEST COMPARISON

Where we win, and where we don't yet

Feature / criteria		Sino AI	DMED	Biofourmis
AI follow-up after discharge	✓	●	✗	✓
Uzbek AI nurse — Telegram & voice	✓	●	✗	✗
Works without a wearable	✓	●	—	✗
Escalation to local patronage nurse	✓	●	✗	✗
Treatment what-if (digital twin)	✓	✗	✗	●
Audit trail that protects clinicians	✓	—	●	●
Own wearable / continuous sensors	✗	✓	—	✓
Large existing user base	✗	✓	✓	✓
Live integration with national / hospital EHR	✗	—	✓	✓
Published clinical outcome evidence	✗	—	—	✓

***Our gaps are on the roadmap: DMED / D-Clinics integration in 2027, device partnerships instead of own hardware, outcome data from the Namangan pilot. Competitor data from public information
● partial · — not applicable / no public info



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BUSINESS MODEL

Price per patient, not per clinic

BASIC

\$2 / patient / month

- ✓ Telegram text check-ins
- ✓ Risk zones & alerts
- ✓ Doctor dashboard

VOICE

\$3.50 / patient / month

- ✓ Everything in Basic
- ✓ AI voice calls in Uzbek
- ✓ For patients without smartphones

PREMIUM

\$5 / patient / month

- ✓ Everything in Voice
- ✓ Digital twin what-if simulation
- ✓ Clinic-branded bot

More revenue layers

-  **Setup fee · \$50–100**
one-time per clinic, waived during pilot
-  **Escalation fee · \$0.50–1**
per red-zone alert actually handled by a nurse or doctor
-  **State & insurers · \$2 PMPM + outcome bonus**
10–15% of savings per avoided readmission (~\$2,000–3,400 each)
-  **Family subscription · \$6 / month**
children pay to monitor a parent — no clinic sales cycle

~\$0.55 cost to serve 1 patient / month

~82% gross margin

Hardware later? device at cost + subscription — never hidden in the software price

Proposed pricing, benchmarked to the RPM industry (\$40–70 per patient / month in the US) and scaled to our market. Readmission cost: Carey & Stefos, Health Care Management Science, 2015.

CLIENTS & SALES FORECAST

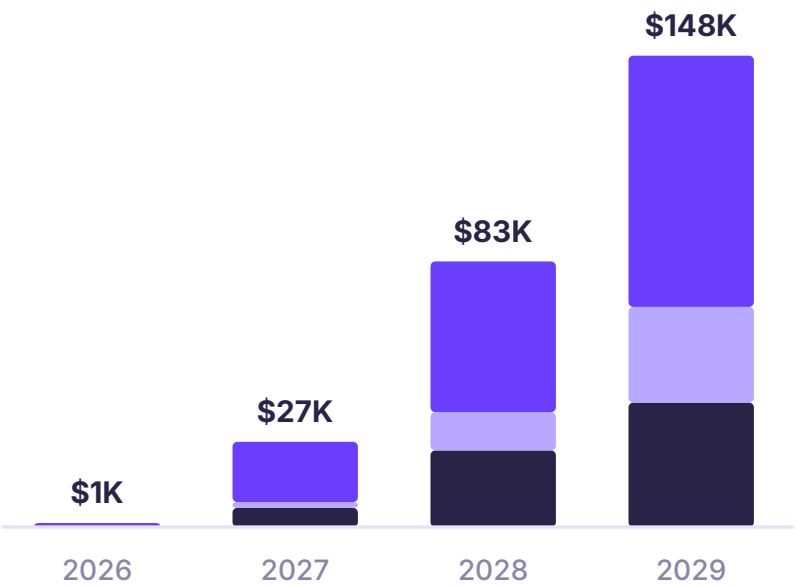
\$1.8M ARR by end of 2029

Monitored patients	2027	2028	2029
 Clinic patients (B2B) private, premium & dental · ~\$3.15 avg	6,000	15,000	25,000
 State patients (B2G) polyclinics & hospitals · \$2 PMPM	3,000	12,000	19,500
 Family subscriptions (D2C) \$6 / month	300	2,000	5,000
MRR (end of year)	\$27K	\$83K	\$148K
Revenue in the year	\$168K	\$660K	\$1.39M
ARR (end of year)	\$0.32M	\$1.0M	\$1.8M

Team estimates for planning — to be validated in the Namangan pilot.

MRR by segment

Clinic State Family



82%

gross margin · ~\$0.55 cost per patient / month

~\$3

CAC per patient (~\$150 per clinic, to be measured)

≈ 2,000

patients to break even (~\$7K MRR) → mid-2027

~7%

of the Uzbekistan market by 2029



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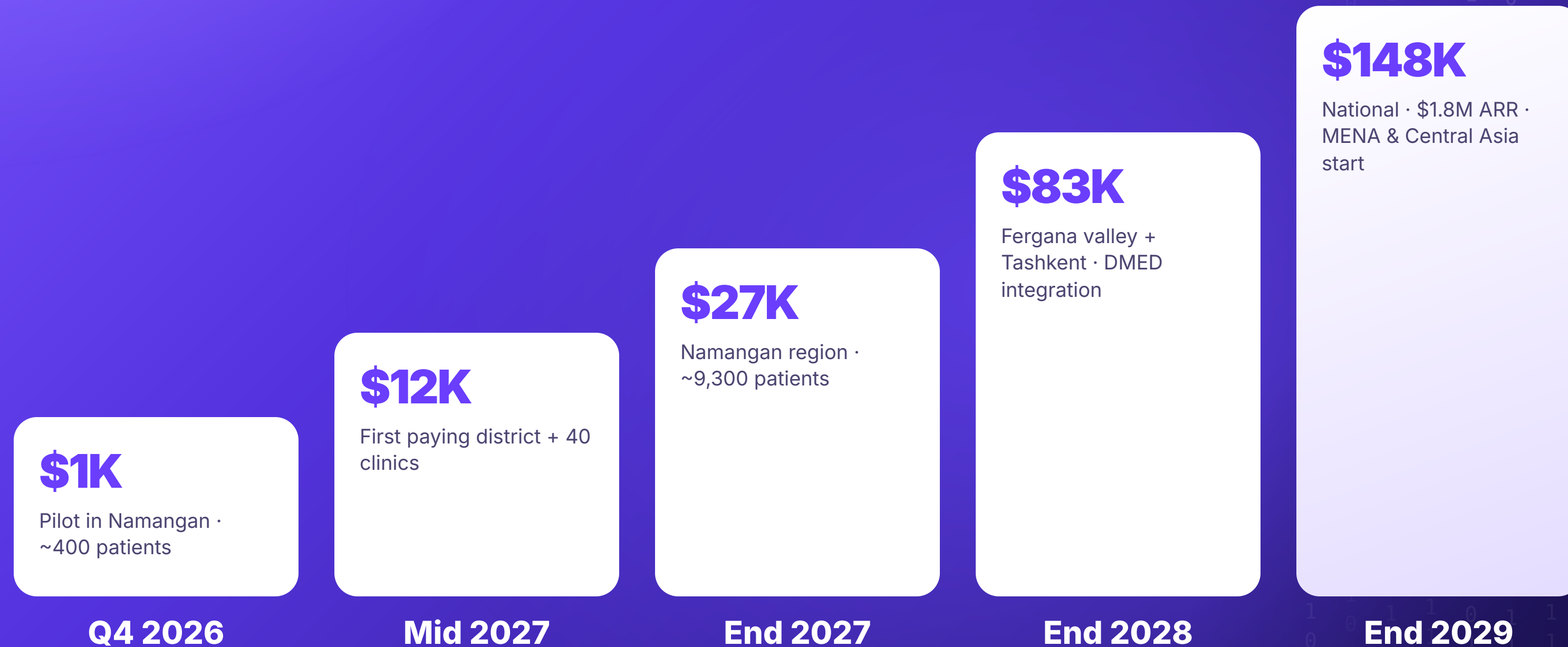
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ROADMAP

MRR targets



THE PEOPLE BEHIND CARE AI

Our team

Product, AI and business in one founding team



Rashitov Abdurashid

CO-FOUNDER & CEO

5 years of experience

Architect, ex-project designer at Uzum Market;
led several projects to growth



Yusupov Rustam

CTO

4 years of experience

Data scientist, LLM & backend engineer · works
at Uzum Holding



Siddikov Nurbek

COO

6 years of experience

Business owner, serial entrepreneur



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Ask

We are raising:

\$20K to run the Namangan pilot and reach break-even within 6 months.

In 6 months:

~2,000 monitored patients

First state pilot results

~\$7K MRR = break-even

\$20K

for 6 months

Use of funds

Product & Uzbek voice AI **\$8K · 40%**

Pilot: 1 polyclinic + 10 clinics **\$5K · 25%**

Sales & clinic onboarding **\$5K · 25%**

Infrastructure & data compliance **\$2K · 10%**

Why now?

9,000+

private clinics — up from ~3,500 in 2017

1.49M

inpatients funded by the new State Medical Insurance Fund in H1 2026

AI speaks Uzbek

LLMs and speech models finally make a local AI nurse possible



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Jury — try the bot

Clinics — let's pilot



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Scan to save our contact



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